



CCPBC

Every child. *Every centre.*

A Pathway to Universal Child Care in BC by 2028

Stabilization

Inclusion

Universal funding

A Pathway to Universal Child Care in BC by 2028

After more than eight years of contract “pilots,” BC still has no stable, inclusive funding model that can deliver universal child care. The province funds a universal “prototype,” but \$10 a day reaches only about one in ten children. Proposed by advocates in 2011 and announced as universal in 2018, it has never come close.

Meanwhile, a significant shift in birth rates and immigration, the normalization of a longer parental leave period, and a large influx of newly built child care mean that operators across the province are reporting vacancies they cannot fill. When children with support needs try to enrol, they are frequently turned away because there is limited or no access to support funding, a gap made worse by the removal of individualized Autism Funding for children under six. Budget 2026 paused new enrolment in \$10 a Day ChildCareBC for a three-year stabilization period.

That pause is the moment to fix the model rather than wait it out.

Child Care Professionals of BC (CCPBC) proposes three linked measures. Each one stands on its own; together they are a path from where the sector is now to a system that serves every family.

1

Stabilize

Pay existing operating and fee reduction funds on 75% of licensed capacity, rather than enrolment, so programs survive the current enrolment trough and are still standing, able to retain their existing educators rather than lay them off.

2

Include

Three changes, none needing new legislation, so that enrolling a child with support needs brings the staffing to provide support within the classroom. A universal child care plan is not universal unless all children, regardless of disability, can access it.

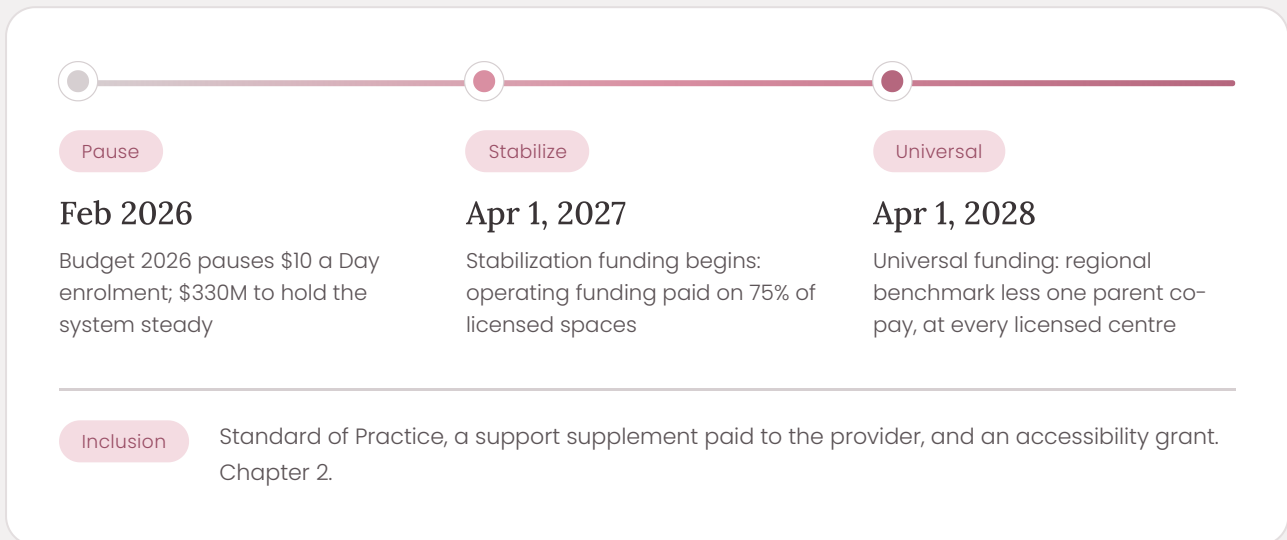
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Go universal

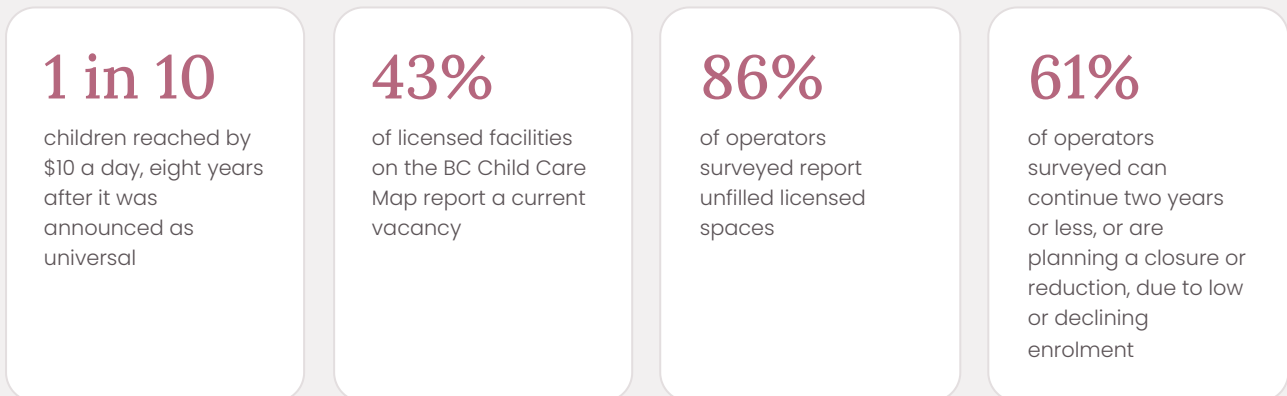
From April 1, 2028, fund every licensed centre at its published regional benchmark, less one province-wide parent co-pay.

The Plan at a Glance

The three measures are sequenced so that no family, operator or educator falls through the gap between today's funding model and the next one.



Stabilization ends when universal funding is in place, or on March 31, 2029 at the latest, whichever comes first.



61%: 90% of these operators report lower 3-to-K or preschool enrolment than in 2022, and 96% report unfilled spaces. BC Child Care Map: 2,567 of 5,991 facilities, September 23, 2026.

Who We Heard From

Between August 27 and September 18, 2026, CCPBC surveyed licensed child care operators across BC about enrolment, vacancies, finances and the future of their programs. Operators responded from across the province: every health authority, all care types and all auspices, including \$10 a Day operators.

42%

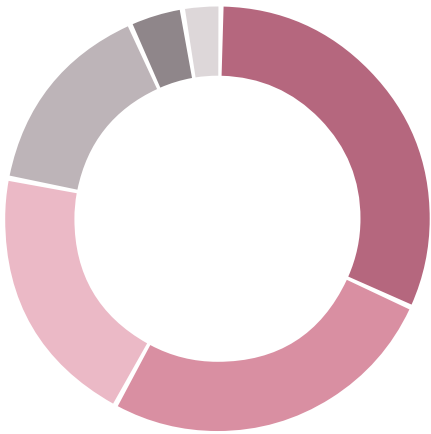
Sole proprietorships

40%

Incorporated companies

18%

Non-profit societies



Operators by health authority

Island Health	32%
Fraser Health	26%
Interior Health	20%
Vancouver Coastal	15%
Northern Health	4%
More than one	3%

Limitations of this survey

Respondents chose to take part, so the results describe the operators who answered, not a random sample of all licensed facilities. Vacancy was reported in bands (for example, “11–25% unfilled”) rather than exact counts. Island and Fraser Health are more heavily represented than Northern Health. Occupancy for the sector as a whole is collected by the Ministry through CCOF and CCFRI reporting but is not publicly available; where this report models costs, it uses published provincial data and states its assumptions.

The Spaces Were Built. *The Children Did Not Arrive.*

BC has spent a decade building child care spaces, and many of those spaces are now sitting empty. Licensed capacity has grown 51% since 2018. Over the same period the demographic base has moved the other way: BC's total fertility rate fell to 1.00 children per woman in 2023, a record low and the lowest in Canada. Slower immigration has removed the offsetting source of young children that historically filled BC programs, particularly in regions with larger immigrant populations.

+51%

licensed spaces since 2018

1.00

BC fertility rate, lowest in Canada

85%

of operators who ran 3-to-K or preschool in 2022 say enrolment is now worse

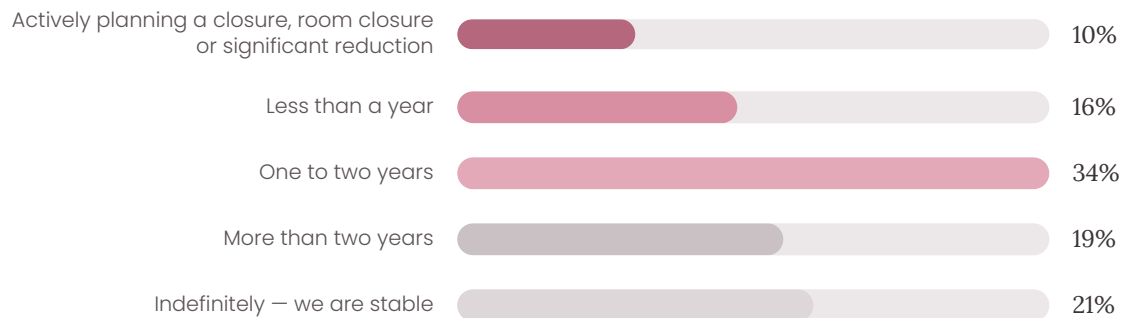
The data used to project the need for new spaces was collected in good faith. But COVID, the current economic downturn, a falling birth rate and an unexpected change in immigration made those population projections inaccurate, at the same time as government invested heavily in new locations. In much of the province, this has created more child care than demand, and in some areas, more child care than children. Pockets of child care deserts remain, but they are no longer the norm.

128

licensed spaces for every 100 children aged three to school age in the City of Victoria. Providers consulted for the City's 2025 Child Care Needs Assessment "noted an oversupply of child-care spaces in the preschool (30 months to school age) category." Infant-toddler access, by contrast, sits at 32 spaces per 100 children.

How Long Can Programs Hold On?

We asked operators how long their organization can continue operating as it is, on its current trajectory. Half said two years or less. Another one in ten are already planning a closure, a room closure or a significant reduction.



BC Child Care Operator Survey 2026.

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We are a community centre. We went from 32 spaces full last year to 26, and now only 8! We have had to lay educators off.

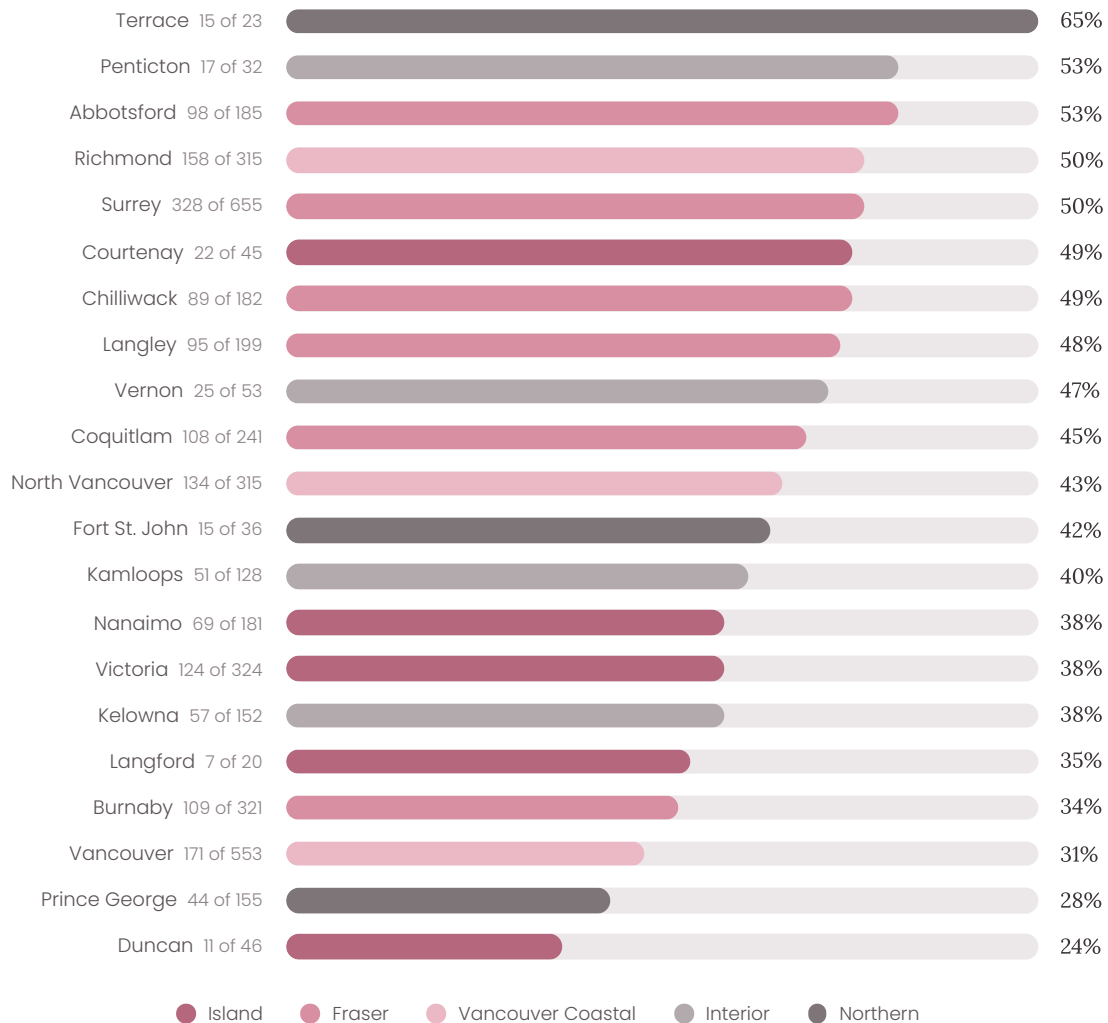
— Non-profit operator, Island Health

Vacancy Across the Province

The BC Child Care Map shows the same picture as our survey. As of September 23, 2026, **2,567 of 5,991 licensed facilities (43%) report at least one vacancy**, and 1,767 report a vacancy for children aged 30 months to school age. Vacancy is highest in Terrace (65%), Penticton and Abbotsford (53%), and Richmond and Surrey (50%).

Share of licensed facilities reporting a vacancy

Largest communities in each health authority

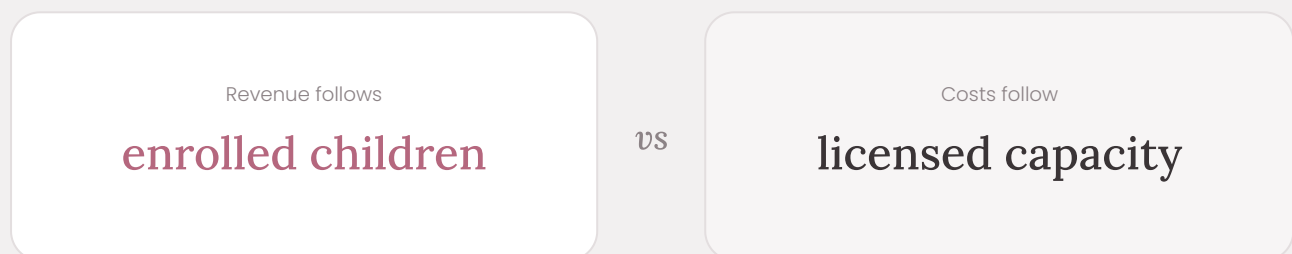


Source: BC Child Care Map Data (Ministry of Education and Child Care), retrieved September 23, 2026, grouped by facility mailing locality. Vacancy is self-reported by each facility and shows whether any space is open, not how many. The published data does not identify health authority, so communities are shown instead.

The Funding Design Turns Vacancy into Closures

CCOF, CCFRI and the CCFRI provider payment are all paid **per enrolled child**. A provider's costs are fixed to **licensed capacity**: lease, licensed staffing ratios, insurance, utilities. A program that loses a quarter of its enrolment loses a quarter of its operating revenue and almost none of its cost.

If the program lays off staff to survive, qualified educators are not available when a family does come looking for a space, creating a practical child care shortage where none existed before.



The ask

Pay CCOF base funding, the CCFRI fee reduction and the CCFRI provider payment on 75% of a facility's licensed spaces, or on actual enrolment, whichever is greater. From April 1, 2027 until universal funding is in place, or March 31, 2029 at the latest.

This is short-term, temporary funding: a bridge to the universal model in Chapter 3, not a permanent change.

Four Safeguards

Four features keep this a controlled, temporary measure, aimed at a sector reporting imminent closures, and not a subsidy for programs that are doing well.

1

A floor, not a ceiling

A program enrolled above 75% is paid on actual enrolment and is better off for every child it adds. The incentive to fill runs in the same direction as it does today; the floor only stops a program falling below the point where fixed costs make it unviable.

2

A three-child minimum

Funding applies only where a facility has three or more children enrolled. In BC, care for two or fewer unrelated children does not require a licence, so the threshold confines stabilization to programs that genuinely need a licence to operate.

3

12 months licensed at that address

The funding is meant to stabilize programs facing an unprecedented decline in enrolment, not to carry a new operator through its first year of business.

4

A fixed end date

It ends when universal funding is in place or on March 31, 2029, whichever occurs first. It is a bridge across a demographic trough and commits no government beyond the current fiscal plan.

No new reporting is needed. Every licensed provider already files monthly enrolment reports with the Ministry, and licensed capacity is on the facility's licence.

What It Costs

Modelled across the 126,624 licensed non-school-age spaces in the Ministry's own CCOF dataset, using occupancy reported in the 2026 BC Child Care Operator Survey.

\$103M

a year: the cost of a 75% floor
\$207M over two years, at most

\$233M

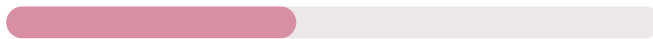
a year in operating funding the system is sized for
but does not pay out, because seats are empty

Operating funding not paid out at
current occupancy



\$233.2M

Cost of a 75% floor



\$103.3M

a year

44%. The request is for less than half of the operating funding the system is sized for but does not disburse. It asks the province to direct money already committed to child care toward keeping the spaces it paid to create.

The cost is self-extinguishing. It is the gap between actual enrolment and 75%, so it falls automatically as children return to the system and reaches zero without any further decision by government. Because the floor only pays where enrolment is below 75%, it targets the programs in trouble and does not overspend on those that are not.

What Stabilization Protects

- **The public investment already made**

Capital grants, New Spaces funding and federal transfers built this capacity. A space lost to closure is not recovered cheaply: the lease goes, the licence lapses, educators are laid off. Rebuilding costs far more than holding.

- **Educator jobs and wages**

Staffing is the first cost a struggling program cuts, and an ECE laid off in a contracting sector is often lost to it for good. Revenue stability keeps educators employed and paid the Wage Enhancement.

- **Infant-toddler programs**

With a group size of 12 and a 1:4 ratio, infant-toddler rooms are costly to build and run, so operators pair them with 3-to-5 rooms. When 3-to-5 rooms empty, infant-toddler rooms are the ones at risk.

- **The runway for universal child care**

A program that closes in 2027 cannot join a universal system in 2028. Holding capacity preserves the inventory universal funding will draw on, and BC's Canada-wide commitments.

- **Choice for shift workers and rural families**

Vacancy is deepest in preschool, family child care and multi-age, which serve part-time families, shift workers, rural communities and children who do not thrive in large groups.

- **Fiscal predictability**

Capped by its own formula, no new administration, a fixed end date, and it shrinks as enrolment recovers.

57%

of operators running infant-toddler care say their 3-to-K or preschool rooms have helped cover its cost.

The Spaces Exist. *The Support Does Not.*

BC currently has the child care spaces to include children with support needs. It does not have the funding or staffing capacity to provide the support children require to safely participate in the program. Families of children with support needs are told there is no room, while 86% of operators report unfilled licensed spaces.

Exclusion here is a funding failure, not a capacity failure.

Share of operators running each program who report unfilled spaces



Data from the BC Child Care Operator Survey 2026.

Support Is Shrinking

At the same time, two sources of support for children with diagnosed or documented needs are contracting.

1

Autism funding is narrowing

Individualized autism funding is being folded into the new BC Children and Youth Disability Benefit, which opened April 1, 2026 and pays \$6,500 or \$17,000 a year to children assessed at the highest level of functional support need. Children are losing their ability to have individualized funding directed to providing individual support in the program. The income-tested supplement meant to sit beneath it does not launch until July 2027, a gap of more than a year.

2

Supported Child Development is rationed

Supported Child Development and Aboriginal Supported Child Development carry long waitlists. Where a placement is offered it is often for a limited block of hours: a family paying for a full day receives support for part of it, and the child attends for as long as the support worker is in the room.

The result is a child care system holding empty spaces it cannot staff, next to families who do not have the funding or ability to provide the staffing themselves.

“

We are struggling to find prompt and adequate support for our children with special needs.

— Family operator, Vancouver Coastal Health

Same Child, Different Funding

The child care special needs supplement pays up to \$150 a month, \$1,800 a year, toward the parent's fee. It is income-tested, so a family above the threshold receives nothing, whatever their child needs. Once that child reaches kindergarten, the same government funds the same child very differently.

Funding per child per year



The difference is not only the amount. School districts receive the money directly and spend it on staffing: education assistants, specialist time, equipment. Child care operators receive nothing they can staff with, so a request to enrol a child who needs support is a request to absorb an unfunded cost.

Operators are left choosing between the child and the budget, with no policy telling them how that choice should be made.

Three Changes

None of these needs new legislation.

1

A Standard of Practice requiring an inclusion policy

The Director of Licensing already uses this instrument, under s. 4(1)(e) of the *Community Care and Assisted Living Act*; the Active Play and Screen Use policies work this way. Every licensed program would hold a written policy for three moments: **enrolment**, when a child arrives with a documented need; **emergence**, when a need becomes apparent after the child has started; and **limits**, when the program cannot adequately support the child, and what it must do before withdrawing care.

Licensed child care is a service customarily available to the public under s. 8 of the *Human Rights Code*, so programs owe a duty to accommodate up to the point of undue hardship, which the provider must prove, not assert. A required policy puts that duty in front of operators and gives families a written process.

2

A support supplement paid to the provider, with no income test

Set it at a level that buys support rather than discounting a fee: in the range of **\$750 per child per month**, roughly a part-time support aide. Pay it to the operator, and keep the existing special needs assessment as the gate. A child's support needs do not change with household income.

3

An annual accessibility grant, open to every licensed operator

A published list of eligible items: physical adaptations, sensory and regulation equipment, communication supports, staff training. Allocated regardless of auspice or funding stream. A child's access should not depend on whether a program is non-profit, private or \$10 a Day.

A 2011 Price in a 2026 Economy

The \$10 a day figure comes from the *Community Plan for a Public System of Integrated Early Care and Learning*, published by the Coalition of Child Care Advocates of BC and Early Childhood Educators of BC in April 2011. It set a vision, not an operating model, and its assumptions have not been updated in 15 years. The number was always the slogan; the goal was quality, accessible, inclusive, affordable care, funded sustainably.

1 in 10

children reached by \$10 a day

38%

rise in Canadian consumer prices, 2011
to 2026

\$276

a month: \$10 a day in 2011, in 2026
dollars

“

We are hearing from families that it's a lottery system. ... We're hearing from operators that we don't have the operating model quite right.

— Lisa Beare, Minister of Education and Child Care, announcing the Budget 2026 pause (Global News, February 18, 2026)

Where Advocates Agree

We stand with the Coalition of Child Care Advocates of BC and ECEBC in calling for universal child care now. We share their goals: universal access, fair wages for educators, inclusive care, and predictable fees at any program. \$10 a day remains worth reaching, but families cannot wait for it one funding round at a time. A single published co-pay delivers affordable care to everyone now, and can be lowered as funding allows.

What this means for parents

One price

known before you join a waitlist, at every licensed centre

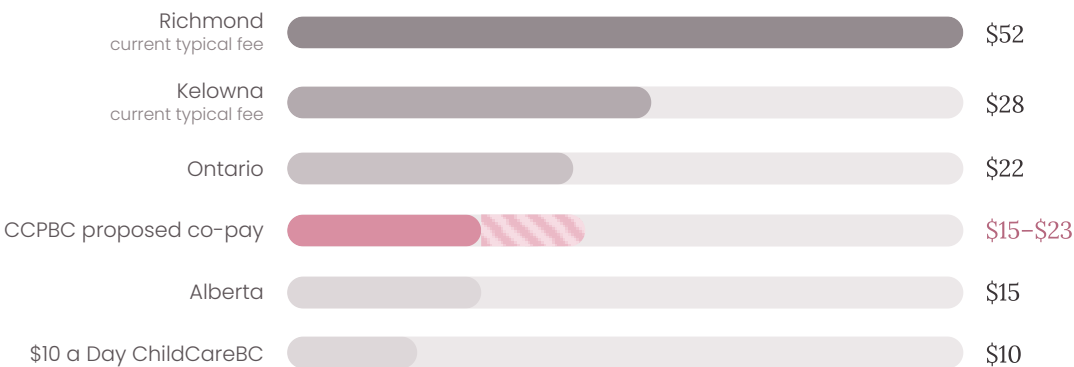
No lottery

no need to win a spot at a designated site; your child stays with their educators and friends

No surprises

if you move, change jobs or have a second child, the price stays the same

Full-time parent fee, dollars a day



Richmond and Kelowna: typical fees reported by CCPA, August 2026. Alberta and Ontario: provincial parent fee levels. CCPBC range: modelled co-pay, see the appendix.

The Funding Model

The work since 2018 did build something more durable than the slogan: fee caps and regional affordability benchmarks. The province publishes a benchmark fee by region and care category, updated annually from data covering more than 90% of facilities. It already governs what new providers may charge. No new rate-setting machinery is needed.



The province funds affordability through four streams today: the Child Care Fee Reduction Initiative (CCFRI), \$10 a Day ChildCareBC, the Affordable Child Care Benefit (ACCB), and Child Care Operating Funding (CCOF). Modelled against current provincial and federal allocations, excluding the ECE Wage Enhancement, we estimate BC could fund every licensed space at its regional benchmark with a parent co-pay of **\$15 to \$23 a day**, comparable to Alberta's \$15 and Ontario's \$22.

The ACCB continues underneath, reducing or eliminating the co-pay for the lowest-income families. The federal government's July 2026 agreement with BC includes more than \$630 million for affordable child care.

This is no longer a budgeting issue; it is a decision. The province can keep funding a 2011 price for fewer than one in ten funded spaces, or use the benchmarks and funding it already has to fund every centre at the cost of care.

Stabilize, Then Go Universal

Phase one · April 2027

Stabilize

Stabilization funding (Chapter 1) keeps operators from closing while the province prepares the transition, and the year is used to settle a transparent benchmark formula with operators.

Phase two · April 1, 2028

Universal

Every licensed centre is funded at its regional benchmark less one province-wide co-pay. Every child, infant to school age, is included: no application rounds, designated-centre lists or lottery.

Universal child care is not universal until every child, regardless of ability, can access it. Inclusion (Chapter 2) is part of the universal model, not an add-on to it.

What this gives each party

Families	A predictable price that no longer depends on whether their centre won a funding round.
Providers	A known benchmark rate, regardless of auspice or funding stream, to plan, budget and set wages against.
Educators	Stability that improves retention, with the ECE Wage Enhancement continuing outside the model.
Government	A model that balances. The co-pay is one lever that can be indexed, scaled by income, or lowered toward \$10 a day as fiscal room appears.
The system	Real universality, so advocacy and policy work can shift to building quality.

Protecting Quality and Accountability

Operators are experts at doing more with less, but quality of care must never become the cost of balancing the budget.

- **Secure and grow the Wage Enhancement**

Protect the existing \$6.00 an hour and add \$2.00 an hour each for ITE and SNE certification, with a publicly available wage grid set by region, developed in consultation that respects collective bargaining rights. Continue the Professional Development Bursary. This is the plan's one new cost; it sits outside the benchmark model and can be phased in over the stabilization period.

- **Set operating-hour ranges**

Preschools run 2 to 4 hours; group care for 30 months to school age runs 5 to 9. Benchmarks reflect the 5-hour minimum and rise proportionately per additional hour; hours beyond 9 may carry a fee. Single-educator programs (IHMA, FCC, MA) cap at 8 hours before fees. Transparent formulas end centre-by-centre contract negotiation.

- **Embed inclusion**

Children with additional support needs are routinely turned away because operators cannot fund support. Chapter 2 sets out the urgent changes.

- **Consult on the benchmark formula**

Use the stabilization year to settle a transparent benchmark formula with operators.

- **Accountability without added burden**

Operators must be accountable through reporting, but accountability measures should not be excessively onerous or add to the current administrative burden. Consider a family registry, similar to the ACCB, where parents register their child and direct their funding to a specific licensed provider, so the Ministry knows immediately which child each payment supports and the room for fraud is removed.

Enough

Operators want to protect the programs families rely on, offer every family affordable care, and keep qualified educators in the field. A funding model that does not reflect the true cost of quality care holds us back, as the province itself has acknowledged. The stabilization period is the moment to fix it.

BC's operators stand ready to deliver a universal system, but we cannot do it alone. We urge the Government of BC to fund every licensed centre at the true cost of quality care, with one published co-pay every family can plan around.

“

If \$10/day is intended to be universal, please actually make it universal so that it is fair for everyone, providers and families alike.

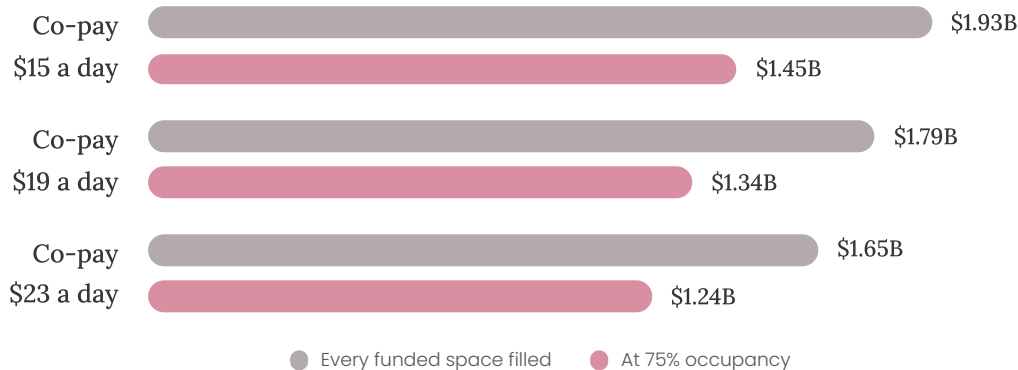
— Family operator, Vancouver Coastal Health

*Every part of this plan comes back to one question:
what is best for the child in the room? The early years
do not wait for the next budget cycle.*

What Universal Would Cost

Funding every licensed space at its regional benchmark, less a co-pay, would cost the province roughly **\$1.2 to \$1.9 billion a year**, depending on the co-pay and occupancy.

Annual provincial cost, \$ billions



Existing streams available to pool: CCFRI \$754.2M and \$10 a Day \$215.9M (2024–25); CCOF (budgeted figures not available). The ACCB (\$134.4M) stays separate, beneath the co-pay, so families most in need keep access to care. The ECE Wage Enhancement stays outside the model.

Assumptions

Spaces: 181,057 funded licensed spaces (BC government, June 2026).

Rates: 2026–27 affordability benchmarks, weighted by each region's share of spaces.

Co-pay: full-day co-pay × 20 days a month for full-time care.

Occupancy: modelled at 75%, the basis of the stabilization formula. Current occupancy ranges from roughly 40–80% by program, based on the BC Child Care Map and the 2026 CCPBC operator survey. The Ministry collects actual occupancy but does not publish it.

Lever that change the cost

Co-pay level: each \$4 a day moves the cost by about \$105M at 75% occupancy (about \$140M if every space is filled).

Payment basis: paying on enrolment rather than licensed capacity.

Sources

Operator survey

BC Child Care Operator Survey 2026, Child Care Professionals of BC (August 27 to September 18, 2026). Occupancy, vacancy, viability and enrolment figures. Quotes are from respondents who agreed to be quoted anonymously.

Chapter 1 · Stabilize

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City of Victoria, Child Care Needs Assessment Update 2025 (Urban Matters and Licker Geospatial Consulting): 127.8 spaces per 100 children aged 36 months to school age; 31.7 per 100 under 36 months. victoria.ca

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Highlighting progress toward ChildCareBC goals: more than 56,000 more children in licensed care than in 2018. news.gov.bc.ca/releases/2025ECC0048-001178

Fertility Trends in British Columbia 1991–2023, BC Stats: total fertility rate 1.00, lowest in Canada.

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Chapter 2 · Include

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Ministry of Education and Child Care, 2025/26 K–12 inclusive education supplemental funding: Level 1 \$51,300, Level 2 \$24,340, Level 3 \$12,300.

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Chapter 3 · Go Universal

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Ministry of Education and Child Care Service Plan 2026/27–2028/29: Child Care line \$947.2M for 2026–27.

Budget and Fiscal Plan 2026/27–2028/29 and Budget 2026 highlights: \$330M ChildCareBC lift; enrolment pause.

CCFRI Funding Guidelines 2026–27: affordability benchmarks by region (Tables 2 and 3).

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R.A. Malatest & Associates, Evaluation and Analysis of ChildCareBC Universal Prototype Sites, 2020.

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